

Standard Operating Procedure

Specimen Collection, Handling, Transport, Reporting and Disposal

«Hospital Name» — Supports AAC.6.e, AAC.6.f, AAC.6.h, AAC.6.j

SOP No.	[Hospital to define]	Version	1.0
Effective Date	_____	Review Frequency	Annual, or on change
Prepared By	Quality Coordinator Name: _____ Signature: _____		
Approved By	Medical Superintendent Name: _____ Signature: _____		

1. Purpose

This SOP describes how «Hospital Name» handles a laboratory specimen from requisition through to disposal, produces standardised reports within defined turnaround time, and manages outsourced testing safely.

2. Scope

Applies to laboratory staff, nurses who collect or transport specimens, and treating doctors who requisition tests.

3. Responsibility

Requisitioning Doctor: orders the correct test with adequate clinical information.

Collecting Staff (nurse/phlebotomist): verifies two identifiers, labels correctly, and follows written collection guidance.

Laboratory In-Charge: defines TAT per test and monitors compliance.

4. Procedure

4.1 Requisition and collection

1. Confirm the requisition names the correct test and carries adequate clinical information.
2. Check two patient identifiers before collection, and again at laboratory receipt.
3. Label the specimen with the patient's unique identification number immediately at collection — never label after the fact.
4. Reject any specimen that is unlabelled, mislabelled, haemolysed, clotted, or insufficient, and log it in the Specimen Rejection Log with the reason.

4.2 Transport and processing

1. Transport specimens per the written guidance for that specimen type, maintaining any required temperature or handling condition.
2. Process within the turnaround time defined for that specific test (per Record 16 — Defined TAT by Test Category). If a delay is likely, inform the requisitioning doctor.

4.3 Turnaround time monitoring

1. Monitor TAT continuously against the defined time for each test category. This hospital's continuous-monitoring approach is adapted from the College of American Pathologists (CAP) Q-TRACKS quality indicator monitoring programme — see Section 8, Reference 1.

2. Escalate any persistent TAT breach to the Laboratory In-Charge and log the reason and corrective action.

4.4 Standardised reporting

1. Every report includes, at minimum: organisation name (or outsourced lab name), patient name, unique identification number, and authorised signatory.
2. Never alter or modify the content of an outsourced report. If issued on this hospital's letterhead, add the outsourced laboratory's name, date, and reference number without changing the result itself.

4.5 Outsourced tests

1. Outsource only to a laboratory with a documented quality assurance system and a current MoU with this hospital.
2. Follow the written outsourcing guidance for packaging, labelling, and transport of specimens sent out.
3. Review outsourced laboratory performance at least annually.

4.6 Disposal

1. Segregate and hand over laboratory waste per the hospital's infection-control / biomedical-waste documents — this SOP does not restate those statutory duties, only requires that they are followed.

5. Records

Supported by Records 1, 3, and 13–30 of the AAC.6 Required Records pack.

6. Related Documents

- HCO_AAC_6_v2_REWRITE_DRAFT — AAC.6 policy
- Hospital infection-control / biomedical-waste management documents

7. Review

Reviewed annually, or sooner when clinical scope, equipment, or outsourcing arrangements change.

8. References

- College of American Pathologists. (n.d.). Q-TRACKS quality indicator monitoring programme. <https://www.cap.org/laboratory-improvement/programs/q-tracks> (Official page; full standard text requires purchase or subscription.)